

إسم المريض رابعياً : كريمة احمد محمد احمد

الحى / المدينة : العمرانيه

المحافضة : الجيزه

رقم البطاقة : 28204272101221 رقم التليفون : 01125463987

العمر(تاريخ الميلاد) : 27-04-1982 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (_____)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان القولون

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان القولون

خطة العلاج : أورام كيمائي خط علاج كيمائي مابعد الجراحه طبقا لبروتوكول (Folfox) [14236]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقيع:

التوقيع:

التوقع:

تعتمد صحة الأيايات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج :
ملاحظة:

مد خطاب : ---

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

ختم

شعار الجمهورية

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Name: كريمة احمد محمد احمد National ID: 28204272101221	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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A patient of colon cancer for Adjuvant chemotherapy according to FOLFOX protocol
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

كريمة
احمد محمد احمد

٣٨ ش سعيد القاضى - العمرانية الغربية
العمرانية - الجيزة

١٩٨٢/٠١/٢٧ ٢٨٢٠٤٢٧٢١٠١٢٢١

IV6690127

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٠١١١١٩٠٦٠٣<

٢٠٢١/٠٣ ٢٨٢٠٤٢٧٢١٠١٢٢١

حاصلة على بكالوريوس دراسات تعاونية

انثى مسلمة

البطاقة سارية حتى ٢٠٢٨/٠٣/٢٨

١

1.5K.

Name	Mrs. Karima Ahmed Mohamed Ahmed		
Ref. By Prof. Dr.		Date	27/4/2026
ID	1126009469	Acc. No.	291331920

1126009469

¹⁸F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of? Pelvic malignancy (as per se). No available previous PET/CT studies or pathological data (patient refused to supply any).

Total body PET scan was done 40 minutes after IV administration of 300 MBq of ¹⁸F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROIs & the normal threshold would be the normal blood pool liver SUV... < 2.3

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain.
(N.B; not all brain metastatic lesions could be detected on ¹⁸-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-/oro-pharynx, larynx, thyroid and salivary glands.
- No enlarged FDG hypermetabolic cervical L.Ns seen.

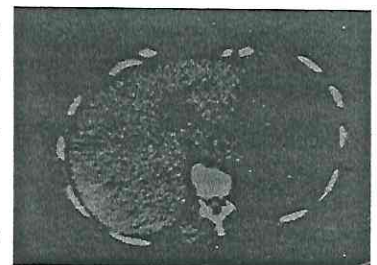
Torso:

Thorax:

- Bilateral tracer starved atelectasis and sub-pleural reticulations.
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.

Abdomen & Pelvis:

- *Average size of the liver with FDG hypermetabolic focal lesions, largest about 4 cm at segment VII eliciting SUVmax. 8.3. For dynamic MRI study of the liver assessment.*
- *Low activity mild ascites with low activity peritoneal folds thickening.*
- No biliary tree dilatation.
- *Metabolically active bilateral heterogenous attenuation cystic adnexal lesions, largest about 21 X 13.5 cm eliciting SUVmax. 3.3.*
- *Left sided bowel loops (presumably distal descending / sigmoid colon) 5 cm segment metabolically active thickening reaching up to 1.2 cm eliciting SUVmax. 16 at ante-mesenteric wall.*



8. COVERED BY D. 1.4

1. COVERED BY D. 1.4

Name: كريمه احمد محمد احمد			
MR. : 10230			
Detailed Medical Report			
			السيد
تحية طيبة وبعد,,			
إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient presented in April 2026 with abdominal distention. Physical examination showed a weight of 76 kg, height 160 cm, and a body surface area of 1.7 m²; dry weight was 69 kg. The patient has moderate ascites and a family history of breast cancer. Imaging revealed bilateral adnexal cystic lesions with a CA-125 of 319. PET-CT on 27 April 2026 demonstrated bilateral adnexal masses, the largest on the left measuring 21 × 13 cm (SUV 3.3), a solitary hepatic focal lesion 4 cm (SUV 8.3), and left-sided bowel loop thickening (SUV 16). The disease was staged as stage IV ovarian cancer versus metastasis of gastrointestinal origin. Multidisciplinary discussion recommended a regimen of Taxol and carboplatin, with plans for endoscopy and ascitic fluid cytology. Baseline labs showed Na 129 mmol/L, K 4 mmol/L, creatinine 2.7 mg/dL, and uric acid 12 mg/dL; the patient was advised close follow-up due to elevated creatinine and uric acid, and carboplatin was omitted for cycle 1 week 1 while Taxol 135 mg was administered. For cycle 1 week 2, Taxol 135 mg was given and carboplatin was again omitted. The patient reported massive ascites requiring therapeutic paracentesis, pending further decision. The pharmacy note requested repeat serum albumin and serum creatinine assessment, and inquired about any known chronic kidney disease. At week 4, the patient had moderate ascites and was evaluated in the pain clinic. Labs were ordered for the upcoming week, including creatinine 1.6 mg/dL and hemoglobin 8.5 g/dL, with a plan to continue Taxol 135 mg and carboplatin AUC 2. During week 5, the patient received Taxol 135 mg, carboplatin AUC 2, and a blood transfusion. On 28 June 2026, the multidisciplinary team decided to pause further treatment until a definitive diagnosis could be established, including immunohistochemistry on a true-cut biopsy of the hepatic focal lesion and colonoscopy. On 20 July 2026, true-cut biopsy of the hepatic lesion revealed mucin-secreting adenocarcinoma favoring colorectal origin. The case was discussed at MDT, and a referral for KRAS testing (result K 2.4) and possible KRAS-targeted therapy was made. On 13 August 2026, MDT planning included placement of a Port-a-Cath and initiation of a Folfox regimen (FOLFOX) with a planned dose reduction of 25 %. On 23 August 2026, the patient experienced an acute right internal jugular vein thrombosis while on enoxaparin (Clexane) and was referred to vascular surgery.			
تحريراً في ,			
التاريخ :		18-09-2026	
الوقت :		dina makram	
الطبيب المعالج :		يعتمد ,	

On 3 September 2026, the patient was noted to have metastatic colon cancer involving the liver and ovaries, with marked ascites and decreased bilateral breath sounds. Current weight was 65 kg (dry weight 53 kg), height 160 cm, and BSA 1.5 m². Baseline labs were obtained. The patient began cycle 1 day 15 of Folfox: oxaliplatin 130 mg IV, leucovorin 600 mg IV bolus followed by 600 mg IV infusion, 5-FU 3600 mg IV infusion, and enoxaparin 60 mg subcutaneously every 12 hours for two weeks. A repeat CT scan was ordered for baseline assessment, and KRAS testing was pending. The patient has now completed one cycle of Folfox.

Name	Mrs. Karima Ahmed Mohamed Ahmed		
Ref. By Prof. Dr.		Date	27/4/2026
ID	1126009469	Acc. No.	291331920

- Average size of the spleen with no focal active lesions.
- Normal appearance of pancreas (no sizable active masses) and adrenals.
- No enlarged FDG hypermetabolic abdominal or pelvic L.Ns seen.
- Congested mesentery with strandy reticulations.
- Normal physiological biodistribution over the urinary tracts.

Surveyed skeleton:

- Faint marrow uptake of SUVmax. 2.5 yet no tracer active lytic or sclerotic osseous lesions seen, likely hematopoietic, for CBC correlation and follow up.
- Scattered sclerotic non-metabolically active foci of the spine and pelvic bones (mostly degenerative changes).

Opinion:

- Active bilateral adnexal and hepatic lesions with low activity ascites and bowel loops thickening (diagnostic possibilities may include metastatic primary ovarian malignancy as per given history with the bowel activity to be mechanical by the virtue of mass effect of the adnexal lesions, however, the possibility of metastatic bowel malignancy still cannot be totally ruled out than otherwise), for unavailable data correlation and histopathological assessment.

Thanks for your referral;

Dr. Ahmed Kandeel
MD Nuclear Medicine

Dr. Islam Aboulwafa, MD
Consultant Radiologist

Date Of Submission:

Date Of MDT:

09/27/2026 10:33:03



Medical Report

Patient Name :

Sex :

☐ Male ☒ Female

كريمة احمد محمد احمد

Age :

44 Y

Comorbidity :

28204272101221

ID No. :

10230

Telephone :

Family History :

Marital Status :

☐ Married ☐ Divorced ☐ Single

Menopausal Status :

Reason of MDT :

ras wild for anti egfr

MDT name::

Medical Report

Diagnosis :

Notes :

History:

met cancer colon (left sided) to ovaries and liver .
initial mri bilateral ovarian massess ascites bilobar hfl
pet ct left bowel loop thickening descending and sigmoid suv 16
hfl biopsy adenocarcinoma grade 2 of colorectal origin
pt had rec 3 weeks taxol carbopl initially before hfl biopsy
then rec up to c2 d1 folfox
ras wild for mdt to add anti EGFR ab

Investigations:

Recommendations:

MDT DECISION:

to continue FOLFOX WITH addition of vectibex
fu on the D1 thrombus

Specialty

Surgery

Pathology

Radiology